

RADIOLOGY REPORT - MRI KNEE

HealthScan Diagnostics Pvt. Ltd.

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Patient Name:	Aarav Sen	Report No:	RAD-2024-08821
Age / Gender:	34 Years / Male	Date:	12-May-2024
Referring Doctor:	Dr. Ramesh Nair, Orthopaedic Surgeon	Modality:	MRI 3.0 Tesla
Study:	MRI Right Knee - Sagittal, Coronal & Axial Sequences		

CLINICAL HISTORY

Patient is a 34-year-old male who sustained a sports injury during a football match approximately 3 weeks prior. Complaints of severe right knee pain, swelling, and inability to fully extend the knee. Physical examination revealed positive Lachman test and anterior drawer sign.

FINDINGS

1. Anterior Cruciate Ligament (ACL):

There is a complete tear of the anterior cruciate ligament (ACL) with disruption of fibres at the mid-substance level. The ligament demonstrates abnormal signal intensity on all sequences. No residual intact fibres are identified. Associated bone bruising is noted at the lateral femoral condyle and posterolateral tibial plateau, consistent with a pivot-shift mechanism of injury. ICD-10: S83.511A - Sprain of anterior cruciate ligament of right knee.

2. Medial Meniscus:

There is a medial meniscus tear involving the posterior horn. The tear demonstrates a horizontal cleavage pattern extending to the inferior articular surface. Grade III signal intensity is observed on proton density sequences. ICD-10: S83.211A - Bucket-handle tear of medial meniscus, right knee.

3. Lateral Meniscus:

Lateral meniscus appears intact with no evidence of tear or degeneration.

4. Posterior Cruciate Ligament (PCL):

PCL is intact with normal signal intensity and morphology.

5. Articular Cartilage:

Mild chondromalacia of the medial femoral condyle (Grade I-II). Patellofemoral cartilage appears preserved.

6. Joint Effusion:

Moderate joint effusion with haemarthrosis noted in the suprapatellar bursa.

IMPRESSION

- Complete tear of the right Anterior Cruciate Ligament (ACL) with associated pivot-shift bone contusions.
- Horizontal cleavage tear of the posterior horn of the medial meniscus.

3. Moderate haemarthrosis.
4. Mild medial femoral condyle chondromalacia.

Clinical correlation recommended. Surgical intervention (ACL reconstruction with possible meniscal repair/resection) is advised.

Dr. Prabhavati Iyer, MD (Radiology), DNB

Senior Radiologist, HealthScan Diagnostics

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